



GLP-1 DIET PLAN

***A practical nutrition, movement and safety guide
for healthy weight loss***



CHAPTER 1

Why Weight Changes After 40

Understanding Menopause, Body Composition, and Muscle

Introduction

Many women say,

"I haven't changed how I eat, but I'm still gaining weight. Others eat less, exercise more, and see little change on the scale."

This isn't simply about willpower. During perimenopause and menopause, hormonal changes affect how your body burns calories, stores fat, maintains muscle, regulates appetite, and responds to exercise.

The goal is not simply to lose weight, it's to lose excess body fat while preserving muscle, protecting bones, and improving long-term health.

Menopause Changes More Than Your Periods

As ovarian function declines, estrogen, progesterone, and testosterone gradually decrease. These hormonal changes affect nearly every organ system and commonly lead to:

- Increased belly fat
- Loss of muscle
- Slower metabolism
- Hot flashes
- Poor sleep
- Mood changes
- Joint aches
- Reduced energy
- Vaginal dryness
- Bone loss

***"Not every woman experiences all of these symptoms,
but they help explain why weight management becomes
more challenging."***

Designed by:

Dr. Shawana Mufti
MD, FACOG





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Why Weight Changes After 40

Hormonal change

Lower estrogen shifts fat storage from the hips and thighs to the abdomen.

This increases visceral fat, the fat surrounding internal organs, which raises the risk of:

- Type 2 diabetes
- Fatty liver
- Heart disease
- High blood pressure
- Chronic inflammation

Your waist measurement is often a better indicator of health than body weight alone.

Muscle loss

Beginning in our 30s, muscle gradually declines, and this process accelerates during menopause. Because muscle burns calories even at rest, losing muscle slows metabolism and makes weight management more difficult.

Maintaining muscle is one of the most important strategies for healthy aging.





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Weight Is Not the Whole Story

The scale cannot tell the difference between fat and muscle.

Two women may weigh exactly the same, yet one has more muscle and less body fat while the other has excess abdominal fat and less muscle.

Healthy weight loss means improving body composition reducing body fat while preserving muscle and bone. Track more than just your weight

- Waist circumference
- Strength
- Energy levels
- How your clothes fit
- Blood pressure and blood sugar (when appropriate)

Protect Your Muscle

Muscle plays a vital role in maintaining a healthy metabolism, supporting strength, and preserving mobility as you lose weight. During GLP-1 therapy, reduced appetite can make it easier to lose muscle along with fat if you don't consume enough protein or exercise regularly. To protect your muscle mass, aim for 100–120 g of protein daily, perform strength training 2–3 times per week, stay physically active every day, and get 7–9 hours of quality sleep. Preserving muscle helps your body burn more calories, improves blood sugar control, supports healthy bones and joints, and makes it easier to maintain your weight loss long-term.

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CHAPTER 2

Starting GLP-1 Therapy

Start low, increase slowly and follow the exact schedule prescribed by your clinician.

What you may notice in the first 12 weeks

Weeks 1-4

Appetite reduces, fullness comes earlier, mild nausea, bloating or constipation may occur.

Weeks 5-8

Healthy food choices often become easier, and weight loss may become more noticeable.

Weeks 9-12

Focus shifts toward muscle protection, fitness and sustainable routines.

Common side effects and simple actions

Nausea

Eat smaller meals, avoid greasy foods and eat slowly.

Constipation

Increase fluids, fiber and daily movement.

Diarrhea

Use extra fluids and simple, easy-to-digest foods.

Heartburn

Eat smaller meals and avoid lying down for 2-3 hours after eating.

Bloating

Slow down and stop at comfortable fullness.

Low appetite

Use small protein-rich meals instead of skipping food all day.





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CHAPTER 3

Your Daily GLP-1 Nutrition Rules

***Because appetite is lower, each meal should
deliver maximum nutritional value.***

Calories

Usually 1,300-1,500 kcal/day, adjusted for body size, activity and medical needs.

Protein

Aim for 100-120 g/day unless your clinician advises otherwise.

Fiber

Aim for 25-35 g/day, increasing gradually if constipation occurs.

Water

Usually 2.5-3 L/day unless fluid intake is medically restricted.





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The 7 golden rules

- Eat protein first at every meal.
- Do not skip meals repeatedly because you are not hungry.
- Fill half the plate with non-starchy vegetables.
- Choose small portions of whole-food carbohydrates.
- Use healthy fats in modest amounts.
- Eat slowly and stop when comfortably full.
- Limit fried foods, sugary drinks, sweets and oversized portions.

Meal rhythm

Aim for three small balanced meals. Add one protein-rich snack only when needed. Large meals are more likely to cause nausea or discomfort.





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CHAPTER 4

The DSM Healthy Plate

Use this structure for lunch and dinner.

1/2 PLATE

Non-starchy vegetables

Salad, spinach, cucumber, tomato, cauliflower,
cabbage, green beans, peppers

1/4 PLATE

Lean protein

Chicken, fish, eggs, Greek yogurt,
paneer, lentils

1/4 PLATE

Healthy carbohydrate

Whole-wheat roti, brown rice, oats, quinoa,
sweet potato, beans

SMALL AMOUNT

Healthy fat

Olive oil, almonds, walnuts, chia,
flaxseed, avocado





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Protein guide

Meal Target Practical examples

Breakfast

25-30 g 2 eggs + Greek yogurt, or protein smoothie

Lunch

30-35 g 120-150 g chicken/fish, or lentils + yogurt

Dinner

30-35 g 120-150 g lean protein with vegetables

Eating technique:

Take small bites, chew thoroughly, pause between bites and stop before you feel overly full.





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When You Wake Up

If your healthcare provider has prescribed medications or supplements that should be taken on an empty stomach, such as thyroid medication or iron supplements, take them first as directed.

Afterward

- Drink 1 cup of warm lemon water to support hydration and digestion.

If You Experience Gas or Bloating

- Add 1/2 tablespoon fennel seed powder (saunf powder) to the water.





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Breakfast (~300-400 kcal)

Choose one of the following options:

Option 1

2 eggs, any style

1 slice whole-grain toast OR 1 small whole-wheat roti

1 cup tea or coffee

Option 2

Greek yogurt

Mixed berries or seasonal fruit

1 tablespoon nuts or seeds





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Breakfast (~300-400 kcal)

Option 3

Oatmeal prepared with milk

1 tablespoon nuts or seeds

Optional berries or seasonal fruit

Option 4

High-fiber, low-sugar cereal

Milk or Greek yogurt

Optional small serving of fruit

Optional

1 small serving of fruit may be added to any breakfast option.





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Convenient Protein Options

If you are short on time, one of the following may be used:

- Protein shake or protein smoothie
- Protein powder mixed with water or milk
- Protein bar, preferably high protein and low sugar
- Aim for approximately 20-30 g of protein at breakfast whenever possible.

Beverage Options

Tea or coffee may be consumed with or without milk.

If desired, use a calorie-free plant-based sweetener such as stevia.

Avoid sugar whenever possible.

DSM Multi-Collagen

DSM Multi-Collagen may be added to coffee, tea, water, smoothies, or warm milk.





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Mid-Morning Snack (~100-150 kcal)

Choose one of the following:

- 1 medium apple + 10-12 almonds
- Greek yogurt + 1 tablespoon mixed seeds
- Seasonal fruit
- Protein shake
- Protein bar, high protein and low sugar

Supplement

Creatine Monohydrate - 5 g daily

- May be mixed with water.
- May be added to a protein shake or smoothie.
- If exercising, it may be taken after your workout.





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Lunch (~350-400 kcal)

Choose one of the following options:

Option 1

120 g chicken, fish, or lean meat

1 small whole-wheat roti OR 1/2 cup cooked rice

Large salad: cucumber, tomato, lettuce, onion, lemon

Option 2

1 bowl daal or beans

1 small whole-wheat roti OR 1/2 cup cooked rice

Large salad or cooked vegetables





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Lunch (~350-400 kcal)

Option 3

Grilled chicken or fish salad

Mixed vegetables

Olive oil and lemon dressing

Option 4

Cottage cheese (paneer) or tofu

Mixed vegetables

1 small whole-wheat roti

Supplements

Omega-3: 1,000-2,000 mg daily

Vitamin D3 + K2 as recommended by your
healthcare provider





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Afternoon Snack (~100-150 kcal)

Choose one of the following:

- 1 fruit
- Greek yogurt
- 10-12 almonds or mixed nuts
- Protein shake
- Protein bar, high protein and low sugar
- Vegetable sticks with hummus

This snack helps maintain energy levels and reduce evening cravings.





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Dinner (~300-350 kcal)

Choose one of the following options:

Option 1

120 g chicken or fish

Cooked vegetables or salad

Option 2

1 bowl daal

1 small whole-wheat roti

Mixed vegetables





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Dinner (~300-350 kcal)

Option 3

Cottage cheese (paneer) or tofu

Cooked vegetables

Option 4

Chicken or vegetable soup

Side salad

Evening Supplement (Optional)

Magnesium Glycinate 200-400 mg before bedtime





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CHAPTER 6

Supplements

DSM Collagen Peptides

Suggested Dose

10–20 g/day

Timing

Morning or evening; mix with coffee, tea, smoothies or water

Benefits

Supports skin, hair, nails, joints and connective tissue.

Protein Powder

Suggested Dose

20–30 g/serving

Timing

Breakfast, after exercise, or between meals

Benefits

Helps meet protein goals and preserve muscle.

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CHAPTER 6

Supplements

Creatine Monohydrate

Suggested Dose

3–5 g/day

Timing

Any time; preferably after exercise or with a meal

Benefits

Supports muscle strength, performance and healthy aging.

Vitamin D3+K2

Suggested Dose

As prescribed

Timing

With a meal containing healthy fat

Benefits

Supports bone, muscle and immune health.





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CHAPTER 6

Supplements

Calcium

Suggested Dose

500–1,000 mg/day if needed

Timing

With meals

Benefits

Supports bone health.

Omega-3 (EPA/DHA)

Suggested Dose

1–2 g/day

Timing

With meals

Benefits

Supports heart health and healthy triglycerides.





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CHAPTER 6

Supplements

Magnesium Glycinate

Suggested Dose

200–400 mg/day

Timing

Evening or before bed

Benefits

Supports muscle, nerve function and sleep.

Vitamin B12

Suggested Dose

500–1,000 mcg/day if needed

Timing

Morning; with or without food

Benefits

Supports nerve function and red blood cell production.

Iron

Suggested Dose

Only if prescribed

Timing

Empty stomach if tolerated or with vitamin C

Benefits

Treats iron deficiency and supports healthy red blood cell production.





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CHAPTER 7

Maintenance, FAQs & Safety

Maintenance principles

- Continue protein, vegetables, hydration and meal planning.
- Keep resistance training and regular activity.
- Monitor weight and waist periodically without obsessing over daily fluctuations.
- Address weight regain early rather than waiting.
- Discuss dose reduction or discontinuation only with your clinician.

If your weight begins to increase, donot wait.

- Early intervention may help prevent significant weight regain.
- A low maintenance dose or individualized microdosing may help maintain your result

Frequently asked questions

How long does it take for my body to adjust to GLP-1 medication?

Most side effects improve within 2–8 weeks as your body adapts. Increasing the dose too quickly may prolong symptoms.





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Frequently asked questions

Is nausea normal?

Yes. Mild nausea is common, especially after starting treatment or increasing the dose.

To reduce nausea:

- Eat smaller meals.
- Eat slowly.
- Stop when comfortably full.
- Avoid greasy or very spicy foods.
- Stay well hydrated.

Seek medical advice if vomiting is persistent or you cannot keep fluids down.

How long do diarrhea or constipation last?

They usually improve within a few days to several weeks.

Helpful tips:

- * Drink plenty of water.
- * Increase fiber gradually.
- * Stay physically active.
- * Discuss medications with your clinician if symptoms persist.





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Frequently asked questions

What is "Ozempic Face"?

Rapid weight loss can reduce fat from the face, making it appear thinner or older. This is not caused by the medication itself, but by losing body fat quickly. Maintaining muscle, eating adequate protein, staying hydrated, and avoiding excessive weight loss may help reduce this effect.

How much weight should I lose each week?

A healthy goal is approximately:

0.5-1 kg (1-2 lb) per week

Rapid weight loss increases the risk of muscle loss, gallstones, nutritional deficiencies and loose skin.

Why did I lose weight quickly at first but now it's much slower?

This is completely normal.

Early weight loss often includes:

- * Water loss
- * Reduced food intake
- * Glycogen depletion

As your body adapts, fat loss naturally slows. Slower weight loss is often healthier and more sustainable.





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Frequently asked questions

What if I stop losing weight?

Weight-loss plateaus are common.

Before increasing your medication, ask yourself:

Am I eating enough protein?

Am I strength training?

Am I walking regularly?

Am I sleeping well?

Am I drinking enough water?

Have my portion sizes gradually increased?

Many plateaus improve after making lifestyle adjustments.

Can I skip meals because I'm not hungry?

No.

Skipping meals repeatedly increases the risk of muscle loss and nutrient deficiencies.

Choose a small protein-rich meal instead.





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Frequently asked questions

Will I regain weight if I stop GLP-1?

Weight regain is common if healthy habits are not maintained.

Long-term success depends on:

- * Nutrition
- * Exercise
- * Muscle preservation
- * Sleep
- * Ongoing follow-up

Medication is only one part of treatment.

What is maintenance dosing?

Some patients remain on a lower dose after reaching their goal weight.

Your clinician may recommend:

- * Continuing the same dose
- * Reducing the dose gradually
- * Increasing the interval between injections (for selected patients)

Never change your dosing schedule without medical advice.





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Frequently asked questions

What is microdosing?

Some clinicians use microdosing smaller or less frequent doses than standard maintenance to help:

- * Control appetite
- * Reduce side effects
- * Maintain weight loss
- * Lower treatment costs

Microdosing is not part of official prescribing guidelines and should only be done under the supervision of an experienced healthcare provider.





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Frequently asked questions

Who should NOT use GLP-1 medications?

GLP-1 medications are not appropriate for everyone.

They should generally be avoided or used only after specialist assessment in people with:

- * Personal or family history of medullary thyroid cancer (MTC)
- * Multiple Endocrine Neoplasia syndrome type 2 (MEN2)
- * Previous serious allergic reaction to the medication
- * Pregnancy
- * Breastfeeding (generally avoided)
- * Severe gastroparesis (delayed stomach emptying)
- * Active pancreatitis or a history of pancreatitis requiring careful evaluation
- * Severe gastrointestinal disease where delayed gastric emptying may worsen symptoms

Use with caution in patients with:

- * Gallbladder disease
- * Advanced kidney disease
- * Significant liver disease
- * Eating disorders
- * Frailty or severe malnutrition

Always inform your healthcare provider about your complete medical history before starting treatment.

